Ysbyty Gwynedd
Anaesthetics Department
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OBSTETRICS

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Anaesthesia Guide For Obstetric Surgery

REVIEW BY JAN 2023

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It is the responsibility of the prescriber to ensure that there are no contraindications to the following:

Premed:

'Pre-Op' carbohydrate drinks, **except for insulin dependent diabetic parturients** (can be taken up to 2 hours prior to surgery):

- For morning theatre cases: 4 cartons at 22:00 on evening prior & 2 cartons at 07:00 on morning of surgery
- For afternoon theatre cases: 2 cartons at 11:30am on day of surgery

Ranitidine 150mg PO 12 hourly on evening prior and morning of day of surgery

Sips of water (no more than 30ml / hour) for the 2 hour period immediately prior to theatre

30ml 0.3M Sodium Citrate PO on arrival in theatre

Anaesthetic:

Regional Anaesthesia (RA):

1. Spinal anaesthesia* using 0.5% Hyperbaric bupivacaine + intrathecal opioid (preferably a long acting opioid e.g diamorphine)

   * 2.5ml Hyperbaric bupivacaine + 300mcg diamorphine is usually recommended for a routine Caesarean Section in a patient of average height

2. Epidural Top up Anaesthesia using 15 – 20ml 0.5% Levobupivacaine plus 100mcg fentanyl

   or 15 – 20ml 2% Lignocaine (+ 1:200,000 adrenaline) plus 100 mcg fentanyl

or General Anaesthesia (GA)

Pre-incision:

Antibiotics: Cefuroxime 1.5G & Metronidazole 500mg (unless previous anaphylaxis to penicillin)

Intra-op:

For RA & GA cases :

Maintain normothermia using an in-line intravenous fluid warmer & ensuring the ambient theatre temperature is 23°C

Avoid excessive intravenous fluid administration

Paracetamol 1g iv (or 15mg/Kg <50Kg)

Dexamethasone 0.1mg/Kg

Diclofenac 100mg PR at the end of surgery if NSAIDS not contraindicated e.g. patient intolerance, pre-eclampsia, HELLP or haemorrhage > 1 litre

For Epidural top up cases only :

Diamorphine 3mg epidurally at end of surgery

For GA cases only :

Maintain adequate muscle relaxation during surgery to optimise surgical access (use TOF monitoring)

Morphine up to 10mg iv in boluses as required

Ondansetron 4mg iv

Bilateral TAP blocks (30ml 0.25% Levobupivacaine each side (or 20ml each side if body weight < 50Kg) to be administered either by the obstetrician during surgery or by the anaesthetist following surgery

Post-op:

For postoperative analgesia advice call the Obs. Anaesthetist (pager 551)

Regular :

For RA & GA cases:

Paracetamol 1g (or 15mg/Kg if body weight < 50Kg) qds PO / iv

Ibuprofen 600mg qds PO

(Avoid NSAIDS in pre-eclampsia & HELLP Syndrome. Prescribe with caution following obstetric haemorrhage. Instead prescribe Tramadol 100mg qds PO)

Plus for GA cases only add:

Oxygen (while on morphine PCA)

Ondansetron 4mg tds iv / im / PO

PRN :

RA cases only:

Oral Morphine Solution 25mg 2 hourly PRN (15mg 2 hourly PRN if opioid sensitive or body weight < 50Kg)

Ondansetron 4mg 8 hourly iv / im / PO

GA cases only:

Morphine PCA (1mg bolus, 5 minute lockout)

Cyclizine 50mg 8 hourly im or iv slowly

Naloxone 100mcg iv up to 4 times at 2 minute intervals

Intravenous fluid :

For RA & GA cases:

Hartmanns solution @ 83ml/hour until adequate oral fluid intake, but needs to continue if morphine PCA connected

To be prescribed by the surgical team : Enoxaparin (if required)

Analgesia TTO for discharge :

Paracetamol 1g (or 15mg/Kg if body weight < 50Kg) qds PO

Ibuprofen 600mg qds PO (substitute with Tramadol 100mg qds PO if NSAIDS contraindicated)